

Program Design of Electronic Information System for Community Elderly Medical Care Optimization

Global Context: Globalization And Sustainability

MYP Community Project 2024-2025

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Introduction

Context

By the end of 2022, the total number of primary healthcare organization in mainland China had reached the number of 980,000. The regions that are covered by community health center is considerably high in mainland China's major urban areas. However, elder residents still tend to visit larger hospitals despite being further away while seeking all types of medical care. This phenomenon has been widely researched and concluded that it may originate from the lack of information of the location and/or other information of nearby health center, which may contribute to the worldwide issue of unequal distribution of medical resources.

This project targets to mitigate the societal issue of uneven distribution of public medical resources particularly due to the senior citizen's lack of information of nearby community health centers.

As an IB learner, our group finds the community project an opportunity to cultivate research, communication and various other skills, while being able to apply knowledge into reality to solve issues. This makes our group aware and familiar of community service and its significance. This project, in our view, is invaluable for becoming a life-long learner.

Our group chose this topic because several reasons. First, we are passionate about helping society to correctly allocate healthcare resources, especially for vulnerable generations like the elderly. The COVID-19 pandemic not only raises notice and concerns toward this topic in our group, but also worldwide. It is especially noteworthy that one of our group members chose COVID-19 vaccine as his PYP-Exhibition theme, which clearly demonstrates our enthusiasm towards this topic.

According to our prior knowledge, despite the high number of community health centers in cities, many residents are not aware of its proximity and the services they offer. Particularly, one of our group members, during the brainstorming, explained that in the region near our school, community health centers are densely distributed, however the large hospital is always extremely crowded, he is convinced that some of the crowd is suitable for visiting community health centers as the community health centers in most time periods are empty. This further convinced us to choose this particular topic.

Additionally, involving the most advanced technologies is especially intriguing for us. In particular, the IoT that we plan to include makes our project more innovative in our opinion, while becoming more likely to create real difference. We believe that by focusing on such fundamental community needs, we can use our full ability to improve healthcare resource allocation that would benefit everyone.

Goal

The goal of our project is to reduce the imbalance in public medical resource allocation, especially due to lack of information to nearby medical services among the older generations.

This attainable goal involves multiple challenging factors combined with in-depth planning, research and knowledge from group members.

First factor is the integration with modern technology, particularly IoT (Internet of Things), which is a newly emerging concept. There is no prior knowledge of this concept, which makes it challenging to research, conclude and finally include it in our specification. However, it is attainable as research and studies are accessible on the internet.

Secondly, the development of application requires basic to intermediate knowledge of programming, which most of our group members are not talented at. On the other hand, this difficulty also involves solutions, as the simple application can include only simple programming knowledge that our group can learn under the time control of around three months.

Furthermore, getting in touch with local health stations and residents to collect data and conduct interviews is not a regular studies skill in school and daily life, which therefore requires good communication, planning, organizing skills concluded from previous activities. However, proper reflection and sufficient time of planning together with support from teachers makes this challenge possible to overcome.

Therefore, while the project's goal is set relatively highly challenging, these challenges are not impossible to overcome with efforts and collaborations.

Global Context

Our global context is globalization and sustainability. It refers to various aspects, primarily the connection of systems created by humans or communities, societal decisions, economic activities and its affections on both human and environment. Physical transportation of human or things, or virtual internet and similar connections are examples of globalization. Meanwhile, sustainability is considered to be related to similar topics: the long-term and current affection of policies in society on multiple aspects, or economic actions for instance the massive construction of factories and its consequences to the environment, etc.

This global context was chosen due to the alignment of our project and the concepts of globalization and sustainability. From the perspective of globalization, our product has a strong correlation with communities. Meanwhile the use of IoT allows our project to connect distinct things in society to serve our targets, hence there is a notable relationship between our project and the global context. Furthermore, the issue that our project is willing to solve, which is the unequal distribution of medical resources, is a crucial

factor in enhancing sustainability globally, particularly in the economic part. Therefore, it is evident that the global context aligns with our project.

This project is significant on multiple levels. On a personal level, this project helps our group to cultivate IB learner skills, including researching, inquiring, communication, etc. It makes our group aware of our responsibility for societal issues like public health.

Meanwhile, being able to access nearby health station information could benefit not only targets in society but also our families. It ensures that they can contribute to a more even allocation on healthcare resources and their experiences on seeking related assistance.

On a broader scale, advancements in healthcare systems can support public health goals. Particularly, it follows China's objective to improve health infrastructure and ultimately the efficiency of societal healthcare systems.

Globally, this project can contribute to the growing awareness of the significance of health care resource distribution, which is critical in achieving a modern concept of sustainable development goals, or SDGs. Finally, this platform can inspire and promote global health equity.

Part A: Investigating

Prior Learning

During our group's studying from grade six to grade nine, we have covered a range of different prior knowledge that contributes to our ability to complete our project. This knowledge can be summarized into five main prior learning:

Firstly, programming language; We have knowledge about the language Python. Python's powerful data processing libraries and abilities can help us carry out the application user interface, visually displaying our idea.

Secondly, geography; Some of the data visualization methods, such as maps and charts, can visually display various data in the application. Coordinates might also be involved in the design of the application.

Thirdly, mathematical statistics; Community health data can be analyzed scientifically and effectively using mathematical statistic knowledge and be displayed through charts such as bar graph and pie charts.

Fourthly, PYP Exhibition; Through the PYP Exhibition and the healthcare-related themes our group members chose, we have learnt about project management, time planning and basic healthcare-related information, which can help to organize and implement community design projects effectively.

Finally, Questionnaire design and analysis; Being familiar with the basic principles and techniques of questionnaire design, we can design a questionnaire that effectively collects the healthcare needs of people in the community. Questionnaire analysis skills help to extract and interpret the opinions and needs of community residents and provide a scientific basis for system design.

Literature Review

According to our prior knowledge and brainstorming, our group has conducted research and concluded two main types of sources, primary and secondary. Primary sources primarily are data from questionnaires, while secondary sources are from websites, journals and essays.

Primary resources, which are first-handed resources that are directly collected from small groups of target audience; thus, it is a precise study. In our conducted research, we have collected 50 responses from a questionnaire mainly about elderly citizen in a local community near Beijing Huijia Private School and their opinions/facts on visiting community hospitals/large hospitals. Some of the responses are instructive and shed light on our topic. Question 4 is a question investigating on the main healthcare demand of local communities. The result showed that 35 out of 50 are chronic disease patient, which requires long-term treatments. Considering the fact that senior citizens generally have mobility issues, this data imply the need of community hospitals in the local community. Question 7 is another main question, which primarily discusses about whether there is a community hospital nearby. Among all the responses, 22, which is

approximately 44% of the responses are unsure or no community hospitals. In fact, in the community where the data is collected, there is a community hospital, thus responses that denied or showed uncertainty demonstrate the lack of information on local healthcare resources, while it is also important to know that the misconception of “nearby” might cause slight inaccuracy in the data. Another example is question 8, where the reasons of not visiting community health stations are listed. 38 of 50 responses states that they are unsure about the service they can make, 32 responses show lack of trust on the hospital’s quality, while 28 of the responses questions the doctor’s abilities. The leading response type further shows the information asymmetry, while the second and third illustrate common lack of confidence on qualities and services in community hospitals. In question 9, the respondents are asked to whether visit or not visit a community hospital in case the hospital fits in their demand. 24% of them, which is 12 out of 50 responses refuses to visit community hospital. One of the reasons can be that the type of healthcare needed is certainly not fulfillable in community health stations, but the others can be the bias on community hospitals. Finally, in question 10, a brief introduction of our community project plan is given to the respondents, and opinion on using it or not to. 82% of the respondents completely agrees to use the project, which shows that under the background of demands of community healthcare, an application that benefits community healthcare and create convenience during the process is highly demanded.

Regarding to secondary resources, most of the research conducted are related to the topic of elder citizen-related health resource allocation between communities and large hospitals, and the societal trend of aging. For instance, in a journal about senior citizen’s decision on hospitals, the author states that 89.2% of the urban elderly choose large hospitals for medical treatment, but thought that hospital fees were too high, suggesting that the urban community health service system was not reasonably utilized and did not achieve the rational application of the principle that minor illnesses should be treated in the community and major illnesses should be treated in hospitals. This shows the research on data related to the distribution and issues between community health station and large hospitals. Another example is our study about aging. An essay published in social science journal argues that aging will become the most dominant trend of population in China for the next 30 years, and by 2050, 38-42% of the population will be aged 60 and over. Meanwhile, demands of elderly citizen on community hospitals are also researched. In one of the studies, the researcher states that most of the patients in primary hospitals are patients with chronic diseases, and the majority of them are elderly, and community health centers have an important role to play because elderly patients have mobility problems, and most of their chronic diseases can be solved in community health centers and health centers. This can be further proven through evidence such as the results of the survey in Jiangmen City, which showed that the rate of chronic disease patients knowing about community health management was 80.62%, and the acceptance rate was 67.05%. However, except for chronic diseases, some portions of senior citizens tend to visit large hospitals more, and this is believed to be not completely

based on facts and evidence, as minor diseases also lead to elderly to visit large hospitals. This can be shown through nationwide research which shows that 49.82% of the elderly have a need for primary health care services, but only 41.95% of them choose to go to primary health care organizations when they have minor illnesses.

Among all the resources listed in this report and used in the research, there are several main resources used. One of them are from the primary resource, while another two are from secondary sources. In the primary ones, question 10 is crucial in determining our topic, as it proves that the local community has demand on our project. In the secondary resources, the second and fifth resource is important. The second resource used shows the rising aging problem and therefore implying the following increase of healthcare needs, while the fifth one shows that due to various reasons, citizens don't choose to visit community hospitals though their demands are theoretically accomplishable in community hospitals, showing the existing problem of healthcare resource allocation deficiency.

In general, most of these resources are relatively useful due to various reasons. Firstly, most of these resources are highly related to our topic, such as aging, health resource inefficient allocation etc. This relation makes these resources evidence and extend to the topic. Secondly, specific resources, such as local questionnaire is unique and can provide accurate ideas about current issues and shared acknowledgement about the topic of community hospitals, giving a deeper insight into our target audience and the theme at the same time.

However, these resources also require an assessment on their quality and reliability. Primarily, our group's source is assessed through a standard called CRAAP test, which stands for Currency, Relevance, Authority, Accuracy and Purpose. For currency, our group have tested all the resources used, most of them are published in 2024, especially websites, which is evidence for a non-outdated information. Secondly, for relevance, our group have carefully checked the articles in the sources, which leaves a relatively relevant source. Meanwhile, most of our sources are from government websites or account, therefore it has a strong authority. Accuracy is also an important criterion of our resources since a larger proportion of them includes citations from other essays/statistics. Finally, the primary purpose of our sources is an inform from government, which further proves the source's quality.

During the research, the librarians are especially helpful. Their assistance in research methods have largely impacted our work, particularly the inform of school's resource in charged academic websites, such as JSTOR, which helped as in searching for information. Meanwhile, except for the guide to owned resources, they also taught us ways of research in the academic websites and remind us of different methods that can be used in reference and specific formats of them.

The whole process involves several categories of skills, specifically inside the bullet points of research skills in the ATL skills. Among them, information literacy skills and critical thinking skills are the

most used skills during our group's investigation. When in the research where large amounts of resources are needed to filter and be used in the proposal or report, both information literacy skills and critical thinking skills are involved to effectively complete it. First, information literacy skills allow us to search widely from different sources and select relevant information. Then, critical thinking skills helps us filter information according to their source, inclusion of data and other critical information.

Part B: Planning

Product/Outcome Specifications

Our product is an electronic information system software design specification. Through this community health management platform, elder citizens can view relevant information and book appointments with community doctors for consultation and treatment anytime, anywhere, get health advice, and integrate various health resources in the community, such as pharmacies, fitness centers, and health seminars, to provide one-stop health services. By designing convenient and easy-to-use health management APPs and smart devices, the elderly can easily carry out health monitoring, online consultation and booking appointments. Additionally, this system can provide personalized health management services by regularly reminding the elderly to undergo health check-ups and take medication. In addition, with the consent of users, by building a community health data platform, health data in the community can be collected and analyzed to help the government and medical institutions formulate more scientific public health policies. Furthermore, it includes the application of IoT and GIS to a certain extent.

A real visit to communities near the school is scheduled, including offer of questionnaires about opinions on the project to residents for feedback collecting, and an interview with local health center workers.

The major inspiration of this project is the extended effects from current worldwide issue of the imbalance allocation and information asymmetry of healthcare resources, especially affecting elder citizens over the age of 60, for instance awareness to proximate medical service stations.

This project will assist on the mitigation of the mentioned issue by providing a solution in the form of integrated platform including information system, geographical information system and data systems that can improve residents' general experience of community healthcare.

Developing Criteria/Specifications for the Product

Specifications for our model includes the following aspects: user-friendliness, compatibility, data security, and functional comprehensiveness. These specifications are based on our understanding of the target user groups, which is elder citizens and our analysis of their demand.

First of all, user-friendliness is one of the main elements of our products. Knowing that our target users are mainly the elderly, who are often less familiar with complex technologies, we will design the interface to be as simple and convenient as possible to ensure that every function can be easily operated. This not only improves the user experience, but also lowers the abilities needed to use the product.

Secondly, compatibility is also an important factor for our products. In the modern world, users access information on multiple devices, particularly, despite being elderly, smartphones and electronic watches such as the Apple Watch are extremely common. According to Hong Kong government's report, more than 65.1% of people aged over 65 owns at least one smartphone, so our system is designed to support and specialize functions for multi-device access, ensuring that users can effectively use our services on any device.

Data security is another key factor. The privacy and sensitivity of health data requires us to strictly focus on Chinese government's data protection regulations and standards in our system design to ensure that user information is in low risk of being compromised or misused.

Finally, comprehensive functionality is an important goal of our product. We not only provide appointment booking and health information viewing, but also integrate functions such as health advice and resource integration, aiming to provide users with a one-stop service that meets their multifaceted needs.

When evaluating our products, we set several success criteria. The first is user satisfaction. We assess user satisfaction with the product through regular user surveys and feedback. Second is feature usage, where we monitor how often each feature module is used to ensure that core features are being fully utilized. Third is system stability, we make sure that the system can run stably even during peak hours and service waiting is kept within reasonable limits. Lastly, we conduct regular security audits to ensure data security.

Plan

Our project timeline is divided into several key phases, each with specific target dates to ensure steady progress. The first phase involves conducting research and gathering necessary data to select a topic and product of the project, which is expected to be completed within the first two months, from 2024. Oct. 9 to Dec. 12. The second phase focuses on preparing and conducting interviews and questionnaires for an actual visit to local communities near Huijia School, targeting completion of investigation and the analysis of data collected by the end of the second week in January 2025, from Dec.12 to Jan.13. The third phase involves creating and refining the product, which is scheduled to take place between January and March, from 2024. Jan.13 to Feb.27. Finally, the last phase is dedicated to finalizing the product and preparing the report, with a target completion date before May 2025, 2025.Feb. 27 to April. 30.

Several challenges are anticipated during the project, some mentioned in the first part of part B in this report. One major challenge is during the creation of our project prototype, the development of the application requires intermediate programming knowledge. However, this challenge can be addressed by dedicating time to learn the necessary programming skills within the given timeframe. Another challenge is collecting data and conducting interviews with local healthcare workers and residents, which requires strong communication and organizational skills.

To ensure the successful completion of the project, our team will focus on effective time management and task distribution. Regular meetings will be held to monitor progress and address any issues. Additionally, our team will utilize self-management skills such as organizational and affective skills to stay on track and maintain an effective use of time throughout the project.

As IB learners, we recognize the importance of mastering the ability to manage our time and tasks effectively. By reflecting on our strengths and weaknesses, we can better distribute our efforts to enhance the weak parts more. Meanwhile, we are confident in our ability to practice time management and self-control during this project. However, we will need to focus on improving communication and organizational skills to ensure the project is completed successfully.

Part C: Taking Action

Achieving Goal

Our final product is diversified. Primarily it is a specification for a community hospital optimization application (The specification is included in the appendices.) Meanwhile, we have created a simple user interface, put up posters on several communities near our school, interviewed residents and community hospital manager about their opinions on our products. We have also created a channel on several social medias to promote public awareness about the convenience and necessity to visit community hospitals.

Our product clearly relates to the global context of globalization and sustainability through our link with the goal of improving the condition of poor allocation of healthcare resources through the promotion of public consciousness on the problem and creating a solution that provide convenience for community hospital service.

To create it, we researched on the status of the aging problem in China, and other issues, including lack of information on nearby community hospitals and the excess preference on visiting large hospitals. We have also researched on the local demands near the school by creating questionnaires and directly interviewing a community hospital manager. We have deeply enhanced the skills of researching and communicating through the above events.

During the creation of the project, we faced challenges. A major challenge we faced was that creating a demo is excessively difficult for us. On IOS systems, a type of language called Swift is used, a language dedicated for user interface building, our group cannot effectively create one by ourselves. We decided to use Artificial Intelligence to help on building the demo application. It turned out to be successful, and the application corresponded the specification.

Except for building the application, the remaining creation of the product is relatively controlled considering the well-developed primary and secondary research on the demands of communities. We are welcomed to promote the awareness of community hospitals across different communities near the school, and most residents accept and seriously taken the interview.

We can conclude that there is a real need for more accessible and well-organized information about community hospitals, and our project is dedicated to assist on this. Although the project is still at a small scale, it demonstrates the potential of combining technology with community outreach to make a meaningful difference.

Thus, we conclude that we are successful in accomplishing our goal because we have successfully created an application solution to help promote awareness of community hospital, assist in better allocation of healthcare resources, while also directly promoting it. We received positive feedback from community hospital workers, and actively promoted the service through social media as well.

Part D: Reflecting

Evaluation of Product

We feel proud of our final product because the product reflects our complete investigation, planning and successful real-world action to improve public awareness and convenience of community hospitals. One of the most positive aspects was the creation of a demo app which integrates maps, calendars, health profiles, chatting interfaces and many other functions. We use the innovative idea of using Artificial Intelligence to create the app completely based on our own ideas in the product specification. Meanwhile, we have also engaged in public promotion through various kinds including brochures, bulletin board posters, and social media. Another notable success is the meeting with a local community hospital manager. We interviewed him twice, with first time understanding the status of local community hospital, and the second time about the functions and feedback of the application.

One challenge we faced was to interview random residents about their idea on the application. Most of the residents does not feel free to be interviewed or have enough time to look through the brochure. However, we did eventually find many willing to participate in the interview.

We met most of our original specifications, both for the project and for the application. For the project, the specification of the application is done according to the schedule. For the application, most of the functionalities and information components are completed. However, due to technical limitations, data security and other complex coding is not completed in the demo. For the plan of the project, the diverse service is not fully explained or considered, but later discovered, thus we added additional contents. Changes could be made at the start, including fully considering the different parts of the project, including researching, service and application design.

We developed new skills throughout the project undertaking challenges of investigating, serving and creating application, including thinking, research and self-management skills. We have improved our communication by interacting with hospital staff and the public. This experience has made us feel more connected to the people around us and more capable of making a positive impact.

Reflections on Knowledge & Understanding

We connected deeply with our global context of globalization and sustainability completing the project by addressing the issue of unequal access to medical resources in large urban areas, specifically in Changping. We better understood how technology like geographic information systems can be used to solve real-world problems, to improve sustainability. By researching about healthcare status, application laws, and gathering community feedback, we gained insight to how global challenges can be identified in

local communities. We also realized the importance of having international mindedness. As learners and global citizens, we have grown more responsible and empathetic. This project showed us how local action together with global thinking, can lead to meaningful and lasting impact.

Reflections on Skills as an IB Learner

After completing this project, we have learned that we are capable to manage complex and meaningful task when we work with a clear goals and collaboration. We developed many key ATL skills, especially in research, communication, and self-management skills. At the beginning, we struggled with organizing large amounts of information, while selecting the meaningful ones with statistics. Time management was also a challenge, especially when we needed to contact community hospitals and prepare for public promotion. However, these skills have improved as we learned to divide tasks, verify sources of information and adjust based on feedback from multiple type of people. By the end, we felt more confident in our ability to communicate with real-world audiences, manage digital tools responsibly, and work as a team. The overall learning process was challenging but rewarding, it helped us grow not just academically, but personally as well.

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Appendices

Figure.1-Bilibili Journal publication



Figure.2-Rednote Journal publication



Figure.4.5-Community Promotion Poster of Beiqijia & Nanshao

99%的人还不知道

开药还能这么轻松！！！！

社区卫生院到底是什么

社区卫生服务中心相关科普

社区卫生院是什么

社区卫生服务中心/社区卫生院是由政府主导设立、服务特定区域的基层医疗机构，主要承担基础诊疗（包括为居民提供一定的药品）、疾病预防、健康管理等任务，覆盖范围通常在街道或乡镇层级，具有便利性高、费用低、服务连续性强等特点，是居民“家门口”的健康守门人。

社区卫生院提供哪些服务

1. 基本医疗服务：包括全科门诊、内外科、儿科、妇科、口腔科等
2. 慢性病管理：筛查、风险评估、定期随访
3. 家庭医生签约服务：涵盖健康评估、体检、个性化健康计划调整
4. 预防保健与公共卫生：儿童疫苗、突发传染病公告等
5. 康复护理服务：术后康复、中医推拿、换药等
6. 中医药特色服务：针灸、膏药贴敷、药浴熏洗等
7. 专家资源与转诊通道：如与三甲医院合作开设专病门诊，专家定期坐诊
8. 便捷化服务：如包括延时门诊（夜诊、周末）
9. 手术服务：如清创缝合、皮脂腺囊肿切除，需配备手术室和麻醉科
10. 健康教育与科普通过短视频、健康讲座等形式普及疾病预防知识

社区卫生院常备药品类型

心脑血管：降压、抗心绞痛、抗血栓药
降糖：二甲双胍、胰岛素等
呼吸：止咳、平喘、感冒药
消化：胃药、止泻、通便药
抗感染：抗生素、抗真菌药
外用急救：皮肤、外伤、急救用药
其他：抗过敏、解热镇痛、慢性病辅助药

附近社区卫生院站点

- ① 翡翠家园社区卫生服务站
- ② 北七家社区卫生服务站
- ③ 冠华苑社区卫生服务站
- ④ 北七佳园社区卫生服务站

99%的人还不知道

开药还能这么轻松！！！！

社区卫生院到底是什么

社区卫生服务中心相关科普

社区卫生院是什么

社区卫生服务中心/社区卫生院是由政府主导设立、服务特定区域的基层医疗机构，主要承担基础诊疗（包括为居民提供一定的药品）、疾病预防、健康管理等任务，覆盖范围通常在街道或乡镇层级，具有便利性高、费用低、服务连续性强等特点，是居民“家门口”的健康守门人。

社区卫生院提供哪些服务

1. 基本医疗服务：包括全科门诊、内外科、儿科、妇科、口腔科等
2. 慢性病管理：筛查、风险评估、定期随访
3. 家庭医生签约服务：涵盖健康评估、体检、个性化健康计划调整
4. 预防保健与公共卫生：儿童疫苗、突发传染病公告等
5. 康复护理服务：术后康复、中医推拿、换药等
6. 中医药特色服务：针灸、膏药贴敷、药浴熏洗等
7. 健康教育与科普通过短视频、健康讲座等形式普及疾病预防知识

社区卫生院常备药品类型

心脑血管：降压、抗心绞痛、抗血栓药
降糖：二甲双胍、胰岛素等
呼吸：止咳、平喘、感冒药
消化：胃药、止泻、通便药
抗感染：抗生素、抗真菌药
外用急救：皮肤、外伤、急救用药
其他：抗过敏、解热镇痛、慢性病辅助药

附近社区卫生院站点

- ① 南部社区服务中心
- ② 北部嘉源服务站

Figure.6-Community Promotion Brochure

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Figure.7-Community Promotion



File.1-Community Hospital System Application Specification

(See Appendix_Specification.pdf)

Figure.8.9.10.11-Investgating

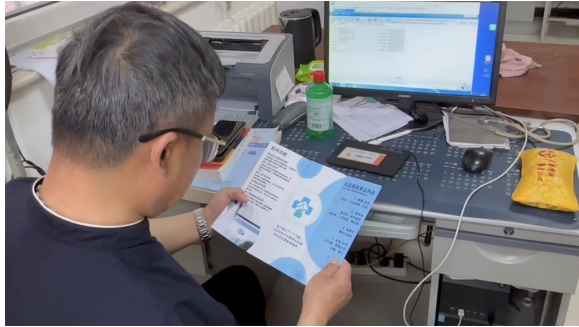


Figure.12.13.14.15.16.17.18-Community Health Management System

社区健康管理中心

系统入口

在线咨询

健康档案

在线预约

医院信息输入

健康档案

个人信息

姓名

张某某

年龄与性别

45岁,男

联系方式

138-1234-5678

医保信息

北京市医保编号: XXXXXXXX XXXXXXXX XXXXXXXX

当前诊断

高血压

糖尿病

健康数据

体重 (kg)

70.5

血压 (mmHg)

120/80

血糖 (mmol/L)

5.6

饮食习惯

低盐低糖饮食

健康目标

减重5公斤, 控制血糖

睡眠状态 (失眠)

无失眠

吸烟/饮酒

不吸烟, 偶尔饮酒

选择医院

